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Received: 3 December 2019 | Revised: 5 June 2020 | Accepted: 22 June 2020

DOI: 10.1111/jpm.12668

LIVED EXPERIENCE NARRATIVE

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A sibling's narrative of recovery from addiction trauma and loss in the family

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Accessible Summary

What is known on the subject?

- Addiction has a devastating impact in the lives of millions of people worldwide. Siblings constitute a hidden population.
- Siblings' attitudes to addiction in the family vary widely from overinvolvement to disengagement among different individuals and across time-line points during the long history of addiction. Female siblings report more negative feelings and attitudes towards drugs and alcohol.
- Previous research has described the process of a sibling's substance abuse through the eyes of a co-sibling but did not focus on how co-siblings are affected personally.

What this paper adds to existing knowledge?

- Addiction can have a damaging traumatic impact on female siblings which is enhanced by social and self-stigmatizing processes related to the "code of silence." The anxiety provoking secret may devour the ability to communicate and connect with other members of their family and community if selective revelation proves to be unsuccessful.
- Ambiguous loss can occur when one's sibling abuses substances, as the sibling may be physically present yet be psychologically absent in terms of the co-sibling (and the family). Ambiguous loss is connected to the lack of understanding of the real impact of addiction on a sibling's personal development.
- Addiction constitutes a family trauma which may severely shake the siblings' beliefs and values and undermine their self-esteem, security and trust in life and the future. Lack of focus on a sibling's feelings during addiction may severely impact on his/her psychological health and identity development.

Implications for practice

- Understanding the experience of siblings may help health and social care professionals to develop compassion, patience and empathy towards family members of persons with addictions. Mental health nurses need to be aware of the devastating impact of addiction on all family members in order to validate their experiences and support them in recovering from trauma and loss in the family.
- Due to the current context of addiction services i.e. individualized approaches and limited resources, organizations should consider the need to provide effective clinical supervision to young mental health professionals at an individual, group or multidisciplinary level.

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KEYWORDS
addiction trauma, biographical narrative interpretive method, loss, recovery, sibling, spirituality

1 | INTRODUCTION

Narratives of genuine honesty are like lamps which can show clearly one's previous life and can be pathways for other people's recovery. Realization of the impact of addiction in a sibling's life can be a torch for other siblings' recovery. It can help them articulate the specific ways in which a siblings' addiction has impacted their lives. The aim of this research was to explicate the experience of a female sibling who has lived through the trauma of family addiction in order to identify the key constituent elements of her lived experience. Five key themes can be identified in the following narrative:

1. Trauma and despair: the perpetual overwhelming anxiety of fearing the worst
2. Ambiguous loss of relationships and healthy family life
3. Family estrangement and social isolation
4. Identity formation and redefinition
5. Gratitude and post-traumatic growth

2 | NARRATIVE

I grew up in a benevolent world with two generous parents who worked as teachers. I remember my mother singing to us and my father narrating Jesus' life stories. During the summer, we spent two months by the sea where all the children of the family were allowed to play barefoot all day long in an atmosphere of freedom. We often had dinner by the sea under the stars and moonlight. On Christmas and Easter holidays, we went to my father's village where I learnt how decent and happy poor people could be. My father (who had lost his father at the age of five and carried a Greek civil war family trauma) and our relatives were all refugees from Turkey and the painful Balkan history of trauma had shaped their philosophy of love, happiness and purpose in life. These wonderful memories were an invaluable treasure of strength for the problems I encountered a few years later.

3 | TRAUMA AND THE PERPETUAL OVERWHELMING ANXIETY OF FEARING THE WORST

During adolescence, my brother was my best friend. I admired him for being tall, good looking and for a blend of sadness and humour in the way he moved around. When he started misusing drugs, I was so shocked that the nucleus of my internal world was shattered. I was totally unprepared for the shock of being so close to a world of users. Why was I so shocked? Why did the world suddenly become so frightening? I followed my brother everywhere obeying my mother's appeal to protect him despite being three years younger than him. I was terrified by the unexpected confrontation with my own inability to answer back to the harsh comments of my brother's friends. I was angry with my parents for not being able to protect us although I had not betrayed my brother's addiction to our parents as he demanded. I didn't know if I was going to wake up one morning only to learn that my brother was dead. When our parents went to our country-house, he would have his friends come over. Our spoons were bent, the smell haunted my mind with memories of his friend's overdose, albeit not fatal, in the bathroom of our flat. In the afternoon, if I was not looking for him, I stayed at home crying because the perpetual anxiety of fearing the worst was unbearable. Worry for his safety contributed to an increased amount of anxious distress with periods of anorexia, depression and irritable bowel disease. Fear haunted my days. I could not assimilate everyday experiences in this malevolent world. The assault of terror was massive. The ever-present threat of catastrophe gradually made me feel a helpless and hopeless spectator in the unfolding drama of my family. Emotional numbness entered my inner life bringing suicidal thoughts as hopeful paths of escaping the unbearable reality. Emptiness permeated my whole being and stripped of the joy and fullness that life has to offer.

Numbing strategies, the emotional first aid against trauma, may help short term but they can complicate the healing process in the long run. Siblings like me who do not fully understand that they have been traumatized are ill-equipped to manage the overwhelming feelings and/or numbing that often occur as a result of addiction. Intense strain or fears of repetition within care taking roles (e.g. as a wife, partner, mother or carer) can often cause further suffering for co-siblings. Indeed, at times of adversity, traumatic memories came back to me. It was as if the past had become present and I was back at the age of sixteen. Trauma was re-enacted. I suffered from flashbacks and nightmares and feelings of "getting lost" in places I knew very well. Still when things overwhelm me, escaping reality might be the only way to avoid the intrusion of painful experiences. This is why I define the lived experience of addiction as traumatic, because it affected dramatically my life and there is still a wound, which although healed through my spiritual practices and personal psychotherapy, it is still painful at times of adversity.

4 | AMBIGUOUS LOSS

I had lost my brother, my best friend and felt very lonely. I was not important to him. Using was more important than me. He was there but still he was someone else, a stranger. I could not recognize the brother I admired. I felt betrayed and rejected.

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Furthermore, the loss of my brother to addiction brought about myriad other losses. The abrupt breakdown of my teenage dreams, the dramatic loss of naivety and teenage illusions was experienced as terror and initiated a prolonged grieving experience. The world seemed malevolent and meaningless. My academic performance was badly affected due to problems of concentration and I seriously questioned my self-worth. I failed my University entry exams. My dream to study Greek literature had vanished forever despite all the awards of the previous years. The process of entering adulthood, developing and maturing involved the loss of a healthy relationship with my brother and my parents, the loss of an older brother who I could look up to and the loss of a healthy family, the loss of friends and normality.

5 | FAMILY ESTRANGEMENT AND SOCIAL ISOLATION

I worried about my brother for years. Preoccupation with the problem brought about anger, bitterness, frustration, exhaustion. I also felt guilty for not talking to my parents about my brother's problems and for sustaining his problems by not intervening. At the same time, I was worried about my parents. I was the protector of my parents believing that the truth would kill them, and at the same time, I was the protector of my older brother whom I would not betray. Because of the stigma associated with substance abuse, I followed the code of silence wherein I did not share my experiences with my parents neither with people outside of the family. Revealing my brother's addiction to members of the shared community would betray “family matters.” The secret however was too heavy to carry it myself. Attempts to talk to some of my beloved friends and teachers were unsuccessful. I felt isolated from my friends and my teachers because of a lack of understanding or the difficulty they had relating to my experience. I got distanced, disconnected and finally cut off from my brother in a desperate attempt to survive, to stop the cycle of my own life's impairment. Ultimately, my sense of connectedness with others was severely impacted by having a substance-using sibling. In the years of my professional life, I often felt alienated, misunderstood and isolated partly due to my tendency to internalize my emotional experiences and the stigma associated with addiction.

6 | IDENTITY FORMATION AND REDEFINITION

A sibling's lived experience can be so extremely devastating that it can interfere with the development of the his/her sense of self and adversely affect the very foundation of his/her personality. It took me many years to recognize the losses associated with the traumatic experiences of addiction in my family. My inner world was destroyed. I lived blind, guided by a nameless traumatizing anxiety. Numbness and repression, shame and secrecy isolated me from others and from myself. My brother's addiction was a powerful, overwhelming emotional experience that changed the very bedrock of my inner world and I became an atheist. The abrupt loss of illusions which were necessary for the smooth transition into adult life made me envious of my friends' gradual experiences of disillusionment as ordinary change. When my brother completed addiction treatment successfully, I helped him with all my heart. Feelings of gratitude paved my way of return to spiritual practices. My postgraduate studies in the UK were a colourful and happy period of my life. During my postgraduate studies, I sometimes discussed my painful story in an emotionally detached, intellectualized manner, often rationalized in one way or another or avoided the traumatic aspects of it altogether by focusing on other less threatening material. Life goes on and I was happy again.

7 | GRATITUDE AND POST-TRAUMATIC GROWTH

Despite having gone through my own personal struggles in life, I was able to become a non-judgemental listener as a wife, mother, colleague and friend, full of respect for the persons asking for my help. My husband recognizes the strength, persistence and patience I had in life. Nonetheless, even today when faced with painful situations, such as chronic rejection, abandonment or shame my body and psyche freeze. It might be that escaping reality is my "default mode" at times of difficulty: Going home to the warmth of my husband and children helps me relax, find myself and the time to be more understanding to my colleagues and other people around me and get off numbness. It's true that I still cannot stand harsh attitudes towards myself. They remind me of the harsh behaviours I met in the drug scene when I was sixteen. I also can't stand harsh attitudes towards homeless people, towards persons with addiction problems, towards their mothers and siblings and all people who were abused and silenced in mute traumas. Yesterday, I explained to my older son why he shouldn't laugh at his classmate who made stupid questions and everybody laughed at her and to my husband who is a teacher why calmer classrooms are essential for abused and/or neglected children.

8 | ETHICAL ISSUES

Ethical approval to share my story was obtained from all members of my family since my narrative constitutes just one version of our shared family history. Therefore, I discussed the content and the purpose of the article with my parents who gave me their blessing to publish it. I also shared the transcript with my brother who agreed to the publication of the article but also wished for the inclusion of his following answer:

“Thank you for sending your narrative to me. There are significant gaps within the narrative but also many leaps that overlook basic principles of a systemic approach to addiction in the family from an early-childhood developmental perspective. In addition,

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addiction 'occurs collectively', no matter which member of the family manifests the problem. Besides my drug use was very light and occasional. The family learned about it much earlier than parents of other families. I have no doubt that the incident was traumatic for the whole family system but there were neither prison nor overdoses. I continued to relate to individuals who did not use drugs and always maintained erotic emotional relationships. I was never alone and never left home. I protected myself relatively effectively, having almost always a sense of self-preservation and boundary. All these, along with the supportive environment of which you were part for some time, pushed me to give up the substances decisively and permanently without any relapses.

In your narrative, the absence of any question about the childhood period is striking. The family system is presented as objectively ideal. For me who lived there, your introduction is a typical example of idealized memory. It presents an ideal family that is suddenly struck by the unexpected illness of addiction on one of its members; that is me. Years of psychotherapy and analysis as well as personal reflection have shown to me that things are quite different. Addiction is a collective state of a pre-existing problem in hypnosis within the system. It does not come in the form of psychopathology that hits one member and then hurts and injures the others. Your text about the personal histories of the family and the system we grew up in is simplistic and ultimately untrue. I understand the need to articulate an adequate and simple narrative with a self-soothing effect. I feel it's a comfortable lie to a certain extent. I'm not talking about the pain and trauma that my addiction and myself may have caused to you, since, as an immature real idiot, I put you manipulatively in my newfound world of substances. I'm sorry for that. I'm sorry I hurt you, as I did my parents. I am also grateful you did not use yourself.

But let's go back to where we started. You present your atheism of a period as a moral derogation. It's a pity that hundreds of millions of people who do not have your own relationship with the 'divine' seem inadequate for your text. I find many problems in your text other than a classification of it as a 'weepy story' etc. Thank you for your courage to show it to me anyway. I remain a loving brother even if he doesn't want a more extensive relationship with you right now. But that's a different matter.

PS.

If anyone asks you again if you have shown the text on your family system, I would not mind if you quoted my present answer. Thanks again. Good night."

9 | AUTHOR COLLABORATION AND PERSONAL REFLECTIONS

Authors had a genuine and equal collaboration. The first author, a sibling, wrote the narrative and the discussion and the second author, a psychiatrist with a group analytical orientation contributed with insightful comments in the structure and content of the discussion section. Both authors strongly believe that a narrative author's first-person perspective does not come out of nowhere. Instead, it develops in an embodied relationship with others and with the environment in a broader social context and it is greatly influenced by the ability to notice one's sensations, emotions, thoughts and inner imagery as they emerge in intersubjective communicative patterns. In essence, writing a narrative is a singular, we-centred shared process and a unique 'embodied' and subjective adventure situated in interpersonal, social and institutional contexts where authors learn about themselves. However, an author's capacity to share the meanings of lived experiences, intentions, feelings and emotions with others in a pursuit of social connectedness is necessarily limited by one's emotional component of empathy which develops early in infancy and by the extent to which mutual resonances are adequately understood.

10 | DISCUSSION

The above narrative constitutes only a glimpse of the trauma and loss of a female non-substance-abusing sibling facing addiction in the family in adolescence and early adult life. More case examples of the unique set of experiences that accompany being a female sibling through the turmoil of addiction in the family can be found in a co-authored article of seven female siblings, one of which is a Psychology Professor at a US University (Howard et al., 2010). Howard et al. (2010) conclude that trauma and silent loss pervades the narratives of female siblings which are the least acknowledged, yet highly affected individuals in the family system.

Smith-Genthôs Logue Low and Hendrick (2017) explored gender differences in co-siblings' positive and negative emotions associated with their sibling's addiction and suggested that siblings further apart in age or who do not share the same gender may be less likely to mirror an older sibling's addiction behaviours, leading to a greater potential for disagreement and future conflict in the sibling dyad. Furthermore, female siblings reported more negative emotions and more negative attitudes towards alcohol and drugs, which may well reflect women's greater (historically gendered) involvement in the emotional work of relationships. This importance of gendered dynamics on understanding the impact of addiction on the sibling sub-system is discussed by Smith-Genthos et al. who suggest that the disproportionate negative impact on women could perhaps be attributed to gendered social roles and a tendency of female adolescents to demonstrate greater affiliative behaviours in relationships. Straightforward descriptions of care taking behaviours of female siblings towards their co-sibling and parents as well as echoes of the sibling roles of hero, lost child and even mascot can be found both in the present narrative and the narratives of Howard et al. (2010). The assumption of such role in the family sub-system, together with the social stigma may account for the devastating sense of trauma and mute loss in the present narrative which is described as so unbearable that placing one's faith in spirituality and personal psychotherapy were the only ways to manage the emotional effects of it.

According to Smith-Genthôs Logue Low and Hendrick (2017) and the authors of the present narrative, the recognition of loss

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can be perhaps best captured by the seminal work of Boss "ambiguous loss" (Boss, 1999). Two fundamental models reside at the core of his theory: (a) physical absence with psychological presence and (b) psychological absence with physical presence. Boss (1999) defines ambiguous loss as "an incomplete, uncertain loss of a loved one, where ambiguity interferes with meaning making, causing lack of resolution" (p. 3). Ambiguous loss is a loss that remains unclear. While communities traditionally comfort those who face clear losses, less attention is paid to ambiguous loss. Yet, the trauma devastates family members' lives because it is incomprehensible, painful and immobilizing. The ambiguity freezes the grief process and prevents cognition, thus blocking decision-making processes. Closure is impossible. Grief is frozen. Family members have no other option but to construct their own truth about the status of the absent person. Siblings cannot name their experience. They need a name which will not pathologize their experience such as "what you are experiencing is an ambiguous loss, the most difficult kind of loss because there is no closure" (Boss, 2010, p.138). They need validation for their feelings of helplessness and sadness as well as hope and meaning behind their chronic experience of suffering. According to Boss (2010), hope lies in discovering that suffering is not an assault on our personal comfort but a process of becoming more spiritual and of discovering patience and forgiveness.

However, although the recognition of loss by the co-sibling of a person with addiction problems can be very potent, yet there must also be a recognition of trauma which pervades the present narrative. According to Smith-Genthôs Logue Low and Hendrick (2017), trauma of non-using siblings can be better understood through the lens of the "loss of illusions" framework developed by Janoff-Bulman and Frantz (1996). "Three fundamental assumptions reside at the core of our inner world: (a) my world is benevolent, (b) my world is meaningful, and (c) I am worthy" (Janoff-Bulman & Frantz, 1996, p. 135). For the author of the present narrative, the assumption of a benevolent world was shattered by the chaos and communication blockage brought about by the stigmatizing secret of addiction which affected severely social support recourses. The meaningfulness of this world was challenged which is also explicated in the lengthy personal accounts of co-siblings contained in the Howard et al.'s contribution to the *Journal of Loss and Trauma* (Howard et al., 2010). Finally, the illusion of self-worthiness was shaken by an adored substance-abusing sibling for whom a caring co-sibling was simply not important when compared to the gratification provided by "using." Life illusions are important recourses of strength for adolescents and traumatized individuals of all ages.

Acknowledging that the theoretical framework for the lived experience of female co-siblings of substance-using individuals could be embedded in the trauma and loss literature has several implications for mental health professionals and addiction nurses. Siblings experiences need to be validated so that denial of their feelings comes to a halt bringing about relief. Development of treatment options and support structures in the addiction field is necessary. However, despite the importance of early interventions in cases of increased isolation and limited connectedness in the social sphere, it appears that it takes years to co-siblings to acknowledge the impact of addiction on their lives and the ways they connect to members of their family of origin and their community. Further research could illuminate the relational patterns of co-siblings who may have a reduced ability to think reflectively about themselves and about relationship experiences. Mental health professionals having a therapeutic relationship should focus on enhancing their ability to reflect on their relationships to other people instead of focusing on recovering memories. Cultivating a trauma awareness culture in the addictions appears to be a core pursuit in this attempt. Routine care interventions should also focus on parental trauma and loss experiences in the face of overwhelming chronic anxiety brought about by the fear of death. Clinical supervision may protect professionals from compassion fatigue and support them in this challenging task.

Furthermore, mental health professionals bring to work their own varied histories, experiences and emotions from their life outside the unit. Oftentimes, their emotional reactions can be partly a result of their own personal history when exposed to the stress and the tensions present in their interaction to traumatized individuals and their families. Therefore, self-awareness, acknowledgement of personal loss history and unresolved issues as well as acceptance of personal limitations constitute necessary equipment in a genuine encounter in the moral space of suffering. It appears that encountering the patient as a 'wounded healer' who has the courage to be himself/herself entails answering questions in oneself related to the meaning of suffering, a process enhancing maturity, knowledge and wisdom. Spirituality could be a precious compass in the long-term journey of resolving feelings of grief and loss at work and of building a strong professional identity.

11 | RELEVANCE STATEMENT

This narrative is relevant to the mental health nursing as its workforce meets numerous siblings and other family members of persons with addictions in their clinical practice. It illuminates psychological processes that can have a devastating impact on the lives of million siblings struggling with addiction problems. The findings have clear implications for the supervision of mental health and addiction nurses who face the challenging task of supporting families in their recovery from addiction trauma and loss.

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